

Incident Reporting Procedure

SECTION 1: PURPOSE

1 PURPOSE

The purpose of this procedure is to ensure that all incidents, accidents, near-misses, and safeguarding concerns are identified, recorded, reported, and managed in a timely, consistent, and transparent manner.

This process protects the health, safety, and rights of all residents, staff, and visitors, and ensures compliance with the Health Act 2007, HIQA regulations, and HSE National Incident Management Framework.

- ✓ Learning is captured to prevent recurrence
- ✓ Open disclosure is practiced in line with ethical standards
- ✓ Incidents are used to drive service improvement and risk reduction

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SECTIONS 2-3: SCOPE & DEFINITIONS

2 SCOPE

This procedure applies to all individuals involved with Chosanta Management, covering all incidents on premises, during off-site duties, online, or involving property, injury, medication, or health and safety risks. Near-misses must also be reported.

- ✓ Full-time, part-time, relief, agency, and volunteer staff
- ✓ Residents receiving care or support services
- ✓ Family members or visitors on the premises
- ✓ Contractors, maintenance personnel, and third-party professionals

3 DEFINITION OF AN INCIDENT

Any event or circumstance that has caused harm, had the potential to cause harm, or interferes with the safety, care, or operations of the service.

Examples of Reportable Incidents:

- ⚠ Injuries: slips, falls, self-harm, assaults
- ⚠ Aggression: verbal abuse, physical aggression, threats
- ⚠ Medication incidents: missed doses, incorrect administration, drug errors
- ⚠ Safeguarding issues: abuse, neglect, exploitation
- ⚠ Environmental risks: fires, floods, electrical faults
- ⚠ Data breaches: lost records, unauthorised access
- ⚠ Missing persons: absconsion, residents not returning from leave
- ⚠ Emergency service call-outs; property damage or theft

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SECTIONS 4-5: RESPONSIBILITIES & PROCESS

4 REPORTING RESPONSIBILITIES

- ✓ **All Staff:** Report any incident or near-miss immediately to the PIC. Reporting is non-punitive — staff will not be penalised for reporting in good faith
- ✓ **Person in Charge (PIC):** Ensures Incident Report Form completed within 1 hour; initiates immediate safety actions; notifies HIQA, HSE, Gardaí if required
- ✓ **Escalation:** If abuse suspected, the Designated Safeguarding Officer (DSO) must be informed. For serious incidents, the Registered Provider is notified

5 REPORTING PROCESS (5 STEPS)

- ✓ **Step 1 — Ensure Immediate Safety:** Provide first aid, remove others from danger, alert emergency services if needed
- ✓ **Step 2 — Notify PIC:** Report to the most senior staff on duty without delay. Also notify DSO if abuse suspected
- ✓ **Step 3 — Complete Incident Report Form:** Within 1 hour — date, time, location, people involved, witnesses, actions taken. Be factual and objective
- ✓ **Step 4 — PIC Logs & Reviews:** PIC checks and signs the report, logs in the Incident Log Register, determines if external notifications required
- ✓ **Step 5 — Escalation & Debrief:** If serious, escalate to management and notify family/next of kin. Arrange debrief and risk review



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SECTIONS 6-7: DOCUMENTATION & NOTIFICATION

6 DOCUMENTATION

- ✓ **Incident Report Form:** Completed within 1 hour — includes date/time, names involved, location, description, actions taken, signatures
- ✓ **Incident Log Register:** Central register maintained by PIC tracking trends, frequency, and investigation outcomes
- ✓ **Notification Records:** Copies of all external notifications retained on file with evidence of submission
- ✓ **Resident File Entries:** Factual entry in individual care record referencing the incident and follow-up support

7 NOTIFICATION

External Notification Requirements:

- ⚠ **HIQA:** Within 3 working days (immediately if serious) — death, serious injury, allegation of abuse
- ⚠ **HSE Safeguarding Team:** Immediately if abuse suspected — physical, financial, emotional, neglect
- ⚠ **Gardaí:** Immediately if crime suspected — assault, theft, missing person
- ⚠ **Tusla:** If a child is involved — allegations or concerns involving minors
- ⚠ **HSA:** Within 10 days if notifiable — dangerous occurrence, major accident
- ⚠ **Next of Kin:** As soon as practical — resident involved in incident or emergency

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SECTIONS 8-10: DEBRIEFING, RECORDS & REVIEW

8 DEBRIEFING & LEARNING

- ✓ Affected staff and residents invited to a supportive debrief focusing on emotional impact, facts, and improvement
- ✓ Learning Reviews for serious/repeat incidents examining root causes and procedural gaps
- ✓ Lessons shared at governance meetings, staff briefings, and annual training updates
- ✓ Outcomes may trigger policy updates, new risk controls, or revised induction training

9 RECORD RETENTION

- ✓ Incident records stored securely in locked cabinets or digital systems with restricted access
- ✓ Records retained for minimum 7 years after case closure; if minor involved, until age 25
- ✓ Records available to HIQA, HSE, or regulatory bodies upon request
- ✓ Quarterly internal audits of incident records for completeness and compliance
- ✓ After retention period, records destroyed securely with destruction log maintained

10 POLICY REVIEW & AUDIT

- ✓ Reviewed at least annually by the Registered Provider and PIC
- ✓ Interim reviews following legislation changes, serious incidents, or audit recommendations
- ✓ Monitored through quarterly audits, monthly management meetings, and staff supervision feedback
- ✓ All staff encouraged to suggest improvements to the reporting process

Incident Report Form

TEMPLATE — COMPLETE WITHIN 1 HOUR OF INCIDENT

Date & Time of Incident:	
Location of Incident:	
Name(s) of Person(s) Involved:	
Name(s) of Witnesses:	
Description of Incident:	
Immediate Action Taken:	
Was First Aid Given? (Yes/No):	
Emergency Services Contacted?:	
Reported to PIC by (Name):	
Date & Time Reported:	
Staff Completing Report:	
Signature:	



PIC REVIEW SECTION

Follow-Up Actions Required:	
Investigation Summary:	
Date of PIC Review:	
PIC Signature:	